

NORTHVIEW REGIONAL HOSPITAL

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PATIENT MEDICAL REPORT

Report Date: 2026-04-22 | Patient ID: NRH-2026-03947

PATIENT INFORMATION

PATIENT NAME	Maria Santos	AGE / GENDER	58 Years / Female
PATIENT ID	NRH-2026-03947	ATTENDING PHYSICIAN	Dr. Kevin Walsh, MD
ALLERGIES	NSAIDs (acute kidney injury)	SPECIALTY	Nephrology

CHIEF COMPLAINTS

- Progressive bilateral leg swelling — 4 weeks
- Foamy urine noticed for approximately 6 weeks
- Fatigue and reduced exercise tolerance
- Loss of appetite; nausea without vomiting
- Known Type 2 Diabetes (10 years) and hypertension (7 years)

VITAL SIGNS

Parameter	Value	Unit	Status
Blood Pressure	162/98 mmHg	mmHg	HIGH
Pulse	78 bpm	bpm	Normal
Temperature	98.8 °F	°F	Normal
Respiratory Rate	17 /min	/min	Normal
SpO2	97%	room air	Normal
Weight	74 kg (+6 kg from baseline)	kg	HIGH

LABORATORY FINDINGS

Test	Result	Reference Range	Status
Serum Creatinine	3.2 mg/dL	0.5 – 1.1 mg/dL	HIGH
eGFR (CKD-EPI)	18 mL/min/1.73m2	> 60 mL/min	HIGH
BUN (Blood Urea Nitrogen)	68 mg/dL	7 – 25 mg/dL	HIGH
Urine Protein/Creatinine	4.8 g/g	< 0.15 g/g	HIGH
Serum Albumin	2.6 g/dL	3.5 – 5.0 g/dL	LOW

Potassium	5.6 mEq/L	3.5 – 5.0 mEq/L	HIGH
HbA1c	9.1%	< 5.7%	HIGH
Haemoglobin	9.8 g/dL	12.0 – 16.0 g/dL	LOW
Phosphorus	5.8 mg/dL	2.5 – 4.5 mg/dL	HIGH

IMAGING & INVESTIGATIONS

Renal Ultrasound	Bilateral small echogenic kidneys (right 9.2 cm, left 9.0 cm); increased cortical echogenicity. No hydronephrosis.
Urine Microscopy	Granular casts present; RBC casts absent. Consistent with diabetic nephropathy.
ECG	Normal sinus rhythm; no peaked T-waves suggestive of hyperkalemia.

DIAGNOSIS

1	Chronic Kidney Disease (CKD) Stage G5	eGFR 18 — pre-dialysis stage; likely diabetic nephropathy
2	Nephrotic-range Proteinuria	Urine PCR 4.8 g/g; hypoalbuminaemia; oedema
3	Renal Anaemia	Hb 9.8 g/dL — normocytic; EPO deficiency
4	Hyperkalemia	K+ 5.6 mEq/L — dietary and renal retention
5	Stage 2 Hypertension	BP 162/98 on current medications

MEDICATIONS & RECOMMENDATIONS

Amlodipine 10 mg	Once daily — calcium channel blocker for BP control
Sacubitril/Valsartan 49/51 mg	Twice daily — ARNi for BP and nephroprotection
Furosemide 80 mg	Once daily — loop diuretic for oedema management
Erythropoietin 4000 IU	Subcutaneous 3x/week — treatment of renal anaemia
Sevelamer 800 mg	Three times daily with meals — phosphate binder
Sodium Bicarbonate 650 mg	Twice daily — correction of metabolic acidosis
Insulin Glargine 18 units	Once nightly — glycaemic management (Metformin discontinued due to CKD G5)

- Urgent AV fistula creation referral — prepare for haemodialysis within 3 – 6 months.
- Strict potassium restriction: avoid bananas, tomatoes, potatoes, dried fruits.
- Fluid restriction 1.2 L/day; sodium < 2 g/day.
- Dietitian referral for CKD-appropriate low-protein (0.6 g/kg/day) diet.
- Diabetes educator for insulin self-management; HbA1c target < 7.5% in CKD.

FOLLOW-UP

Follow-up Date	2 weeks	Tests Required	Creatinine · eGFR · K+ · Hb · BP · Urine PCR
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*Digitally prepared by Dr. Kevin Walsh, MD · Nephrology
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